Valley Regional Hospital - Department of Clinical Services

CLINICAL DOCUMENTATION — NARRATIVE EMERGENCY OBSERVATION NOTE V1

Patient: Linda Davis | **MRN:** PAT-60008 | **DOB:** 1984-12-01 | **Gender:** Female | **Doc ID:** NARRATIVE_0008
Encounter Date: 2025-03-29 | **Discharge/Note Date:** 2025-04-03 | **Provider:** Dr. Lisa Cuddy, MD

EMERGENCY DEPARTMENT PRESENTATION

Patient Linda Davis was triaged in the emergency department following acute onset of lightheadedness and palpitations earlier this morning. Initial hemodynamic parameters were reviewed and patient was placed on telemetry.

HISTORY OF PRESENT ILLNESS

The patient describes experiencing recurrent lightheadedness over the preceding 48 hours without syncope. Initial emergency workup revealed that initial potassium was measured at 3.71 mmol/L. Point-of-care laboratory screening additionally noted alanine aminotransferase (alt) resulted at 27.7 U/L. Given these findings, the patient was transitioned to the Clinical Decision Unit for 24-hour observation.

OBSERVATION UNIT CLINICAL COURSE

Throughout the observation unit stay, continuous telemetry showed normal sinus rhythm without pause or tachyarrhythmia. The patient was administered Azithromycin at a dose of 250 mg oral once daily for 5 days, which promptly alleviated acute symptoms. Follow-up protocol monitoring of platelet count demonstrated 109.4 10*3/uL (LOW). Orthostatic blood pressures were negative upon ambulation.

DISCHARGE REGIMEN

Patient was deemed safe for outpatient discharge. The ongoing regimen includes Omeprazole 20 mg PO once daily in the morning for 30 days as prescribed. Prescription counseling was performed by nursing prior to release.

PLAN & PRECAUTIONS

Resume normal daily activities as tolerated. Avoid strenuous exercise until cardiology follow-up in 1 to 2 weeks. Immediate emergency precautions detailed.